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Letters: When Food Is Modified

To the Editor:

The column "Gene Altered Foods: A Case Against Panic" (Dec. 5) tried to make the case that unease about medical and agricultural uses of technology is often due to inaccurate terminology and poor understanding of science.

Unfortunately, it adds to the confusion by mischaracterizing the nature and role of genes. A plant breeder is quoted on the introduction of a gene from the Arctic flounder into strawberries and cites his interpretation: it is "not a flounder gene but a cold tolerance gene."

But there is no such thing as a cold tolerance gene independent of the organism in which it acts. Biologists agree that genes are context- dependent and may do different things in different cells and tissues of the same organism, not to mention in organisms as different as flounders and strawberries.

I suggest that inaccuracies in scientific terminology and concepts are often due to promoters of new technologies.

DR. STUART A. NEWMAN
Valhalla, N.Y.

The writer is a professor at New York Medical College .

To the Editor:

The Personal Health column on genetically modified foods promotes the misconception it warns of. The portrayal of current genetic "engineering" as precise and well defined is inappropriate today.

Few genes are "known quantities" and the process of introducing a foreign gene into an organism produces uncertainty about both the gene's function and the function of the DNA into which it is inserted.

Genetic engineering techniques are abysmally primitive, akin to swapping random parts between random cars to produce a better car. Yet our ignorance will fade; biological engineering will become a reality relatively soon.

But it is difficult to discuss this impending development when the public believes that the details are already understood, especially when mistakes are so publicly discussed. The conflation of "engineering" and such failures can only suggest a subtext that the problem is beyond hope and that further work will produce dire consequences.

ROB CARLSON
Berkeley, Calif.

The writer is a research fellow at the Molecular Sciences Institute.

To the Editor:

The Personal Health column on Dec. 5 correctly notes the potential of genetically improved food to help feed the world and reduce the need for pesticides. But many are needlessly concerned about negligible risks posed by this technology. Genetically engineered food is thoroughly regulated by at least three federal agencies.

Superstition and fear should not interfere with this technology, which has so much to offer those who suffer from hunger and malnutrition. Unfounded concerns about hypothetical risks are far outweighed by the real benefits that will soon be realized, if scientific research and development of genetically modified agriculture is allowed to proceed unhindered.

DR. GILBERT ROSS
New York

The writer is the medical director of the American Council on Science and Health, a group financed by foundations, trade associations, companies and individuals .

Debating Prognosis

To the Editor:

In February, my wife was found to have cancer in her breasts, lungs, stomach, liver and bones, so the Conversation With Dr. Nicholas Christakis on Nov. 28 hit home ("A Doctor with a Cause: `What's My Prognosis?' ")

In May, we received conflicting recommendations about chemotherapy. So we wrote to the doctors involved asking for a list of options and the estimated prognosis — length and quality of life — for each. That letter was never answered, and my wife chose not to proceed with any treatment option without that information.

Later we learned that one doctor "could not guess" the prognosis and had recommended chemotherapy lest I think that not enough was being done. I replied, "I wish I had been asked."

After my wife entered home hospice care in August, I asked the nurse how long she might live and was told "about four weeks." She died three weeks and three days later, without significant pain, at home, surrounded by family.

We were not seeking miracles or reassurance, just information.

Dr. Christakis is quoted as saying, "In those cases where patients ask, I will tell them." Amen!

ROBERT F. ROSIN
Fair Haven, N.J.

To the Editor:

Re "What's My Prognosis?": Dr. Nicholas Christakis laments the neglect of prognosis; I deplore the overzealousness of some doctors in predicting outcomes. In April 1998, I received a diagnosis of terminal cancer. My husband asked my prognosis, and we were told, "at most, nine months."

The doctor said my cancer was neither curable nor treatable. When pressed, he said that chemotherapy might add a couple of months but that the side effects would be debilitating. We agreed it wouldn't be worth it.

I began to make arrangements for dying, seeing no reason for doubt. Fortunately, others did and urged me to seek other opinions and treatments.

I saw another doctor who said things could be done. I am alive and doing well 19 months later.

I recognize the issue's complexity. Dr. Christakis correctly points out that if the patients think they are going to live a year when, in fact, they are going to live a month, different choices will be made.

The converse is also true. If you believe that you are going to live a month, instead of years, there is no reason to pursue treatments that may prolong your life. Which mistake is worse?

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